

CUSTOMER JOURNEY MAP

Medical Diagnosis Center by Service Portal

ServiceNow | Patient Diagnostic Service Journey | 06 October 2026

Purpose

This document maps the complete patient journey from identifying a diagnostic need through service discovery, booking, validation, fulfilment, report communication, acceptance, production launch and continuous improvement.

1. JOURNEY FRAMEWORK

The structure follows the uploaded reference journey map: Entice, Enter, Engage, Exit and Extend, with the supporting dimensions of Steps / Experience, Interactions, Goals & Motivations, Positive Moments, Negative Moments and Areas of Opportunity.

ENTICE	Become aware of the need	Stage 1
ENTER	Find and configure the service	Stages 2–3
ENGAGE	Submit, approve, fulfil and communicate	Stages 4–7
EXIT	Validate and gain acceptance	Stages 8–9
EXTEND	Launch, monitor and improve	Stages 10–11

Journey sequence

1. Need Identified → 2. Service Discovery → 3. Test & Appointment Configuration → 4. Submit & Validate → 5. Approval & Governance → 6. Test Fulfilment → 7. Notifications & Status → 8. Testing & Validation → 9. UAT & Acceptance → 10. Production Launch → 11. Monitor, Support & Improve

Design principle: Each patient-facing moment is connected to a ServiceNow behaviour, an owner/handoff, a success signal and an improvement opportunity.

1. Need Identified | ENTICE

STEPS / EXPERIENCE	A patient identifies the need for a diagnostic test, appointment, follow-up, or report.
INTERACTIONS	Service Portal / browser
PRIMARY OWNER / HANDOFF	Patient / caregiver
GOALS & MOTIVATIONS	Recognizes a clear healthcare need and knows where to start.
POSITIVE MOMENT	Clear entry point for diagnostic services.
NEGATIVE MOMENT	Patient is unsure which service or test is required.
AREA OF OPPORTUNITY	Use a clear portal entry point and simple diagnostic service categories.

2. Service Discovery | ENTER

STEPS / EXPERIENCE	The patient enters the Service Portal and explores available diagnostic services.
INTERACTIONS	Service Catalog / search / categories
PRIMARY OWNER / HANDOFF	Patient / service coordinator
GOALS & MOTIVATIONS	Finds the correct diagnostic test or service quickly.
POSITIVE MOMENT	Relevant test categories and concise descriptions.
NEGATIVE MOMENT	Too many choices or unclear categories can cause wrong selection.
AREA OF OPPORTUNITY	Use search, filters and categories such as Pathology, Radiology and Cardiology.

3. Test & Appointment Configuration | ENTER

STEPS / EXPERIENCE	The patient selects the test, appointment date, time slot and required details.
INTERACTIONS	Catalog item / variables / dynamic fields
PRIMARY OWNER / HANDOFF	Patient / receptionist
GOALS & MOTIVATIONS	Provides complete booking information with minimal effort.
POSITIVE MOMENT	Simple, guided booking form.
NEGATIVE MOMENT	Long forms, unclear fields or missing guidance create rework.
AREA OF OPPORTUNITY	Use dynamic fields, mandatory validation and prescription upload where applicable.

4. Submit & Validate | ENGAGE

STEPS / EXPERIENCE	The patient reviews and submits the request; the platform validates data and slot availability.
INTERACTIONS	Validation rules / booking record / platform
PRIMARY OWNER / HANDOFF	Patient / platform automation
GOALS & MOTIVATIONS	Creates a valid booking without duplicate or conflicting appointments.
POSITIVE MOMENT	Immediate confirmation with appointment/reference details.
NEGATIVE MOMENT	Incomplete data or unavailable slots can delay booking.
AREA OF OPPORTUNITY	Implement real-time slot validation, duplicate prevention, capacity limits and booking rules.

5. Approval & Governance | ENGAGE

STEPS / EXPERIENCE	Special or high-value diagnostic tests are routed for approval when required.
INTERACTIONS	Flow Designer / approval record / audit
PRIMARY OWNER / HANDOFF	Lab manager / administrator
GOALS & MOTIVATIONS	Approval decision is clear, traceable and auditable.
POSITIVE MOMENT	Approval ownership is clear and traceable.
NEGATIVE MOMENT	Approval ambiguity or missed notifications can delay specialized tests.
AREA OF OPPORTUNITY	Define approval rules, escalation and audit history.

6. Test Fulfilment | ENGAGE

STEPS / EXPERIENCE	Confirmed bookings are converted into actionable tasks for reception, sample collection, radiology or laboratory teams.
INTERACTIONS	Fulfilment task / assignment / patient context
PRIMARY OWNER / HANDOFF	Lab technician / diagnostic team
GOALS & MOTIVATIONS	Test task is completed with clear ownership and complete patient context.
POSITIVE MOMENT	Lab team receives complete, actionable test information.
NEGATIVE MOMENT	Manual handoffs or incomplete information can cause rework.
AREA OF OPPORTUNITY	Automate task creation and assignment using Flow Designer.

7. Notifications & Status | ENGAGE

STEPS / EXPERIENCE	The patient receives appointment, sample, processing and report-status updates.
INTERACTIONS	Email / portal status / request timeline
PRIMARY OWNER / HANDOFF	Patient / lab team / support
GOALS & MOTIVATIONS	Patient can understand progress without repeatedly contacting the center.
POSITIVE MOMENT	Patient can track progress without repeated calls.
NEGATIVE MOMENT	Silence or technical messages increase patient uncertainty.
AREA OF OPPORTUNITY	Use clear lifecycle statuses and event-based notifications.

8. Testing & Validation | EXIT

STEPS / EXPERIENCE	The configured journey is tested before release or after major changes.
INTERACTIONS	Test scripts / logs / test records
PRIMARY OWNER / HANDOFF	Developer / tester / service owner
GOALS & MOTIVATIONS	Critical booking, workflow, notification and access paths pass testing.
POSITIVE MOMENT	Defects are found before patient-facing release.
NEGATIVE MOMENT	Uncovered edge cases can cause failures after release.
AREA OF OPPORTUNITY	Test normal, negative, cancellation, rescheduling, duplicate and access scenarios.

9. UAT & Acceptance | EXIT

STEPS / EXPERIENCE	Healthcare and operational stakeholders validate the complete experience against realistic scenarios.
INTERACTIONS	UAT scenarios / evidence / acceptance record
PRIMARY OWNER / HANDOFF	Patient representatives / lab staff / admin
GOALS & MOTIVATIONS	Acceptance criteria are satisfied before production.
POSITIVE MOMENT	Stakeholders validate realistic healthcare scenarios.
NEGATIVE MOMENT	Late feedback or unclear acceptance criteria can extend delivery.
AREA OF OPPORTUNITY	Use representative patient, booking, report and approval scenarios.

10. Production Launch | EXTEND

STEPS / EXPERIENCE	Approved configuration is migrated to production and production behaviour is verified.
INTERACTIONS	Deployment checklist / Update Set / training
PRIMARY OWNER / HANDOFF	System administrator / release owner
GOALS & MOTIVATIONS	Production validation succeeds with controlled release.
POSITIVE MOMENT	Launch is controlled and prepared.
NEGATIVE MOMENT	Configuration gaps or unprepared teams can create launch friction.
AREA OF OPPORTUNITY	Use controlled migration, smoke testing, training and rollback/support planning.

11. Monitor, Support & Improve | EXTEND

STEPS / EXPERIENCE	The live service is monitored and feedback is converted into continuous improvement.
INTERACTIONS	Dashboards / metrics / audit / feedback
PRIMARY OWNER / HANDOFF	Support / administrator / service owner
GOALS & MOTIVATIONS	Operational trends and feedback drive measurable improvement.
POSITIVE MOMENT	Service becomes measurable and continuously improvable.
NEGATIVE MOMENT	Untracked defects, weak metrics or feedback gaps can hide recurring problems.
AREA OF OPPORTUNITY	Track bookings, demand, report turnaround time, cancellations, incidents and feedback.

3. ENHANCED JOURNEY INTELLIGENCE

This layer connects the patient journey to the backstage ServiceNow configuration, ownership and measurable success signals.

JOURNEY STAGE	BACKSTAGE SERVICENOW EXPERIENCE	PRIMARY OWNER / HANDOFF	SUCCESS SIGNAL
Need Identified	Portal entry and diagnostic service taxonomy are prepared.	Patient / Service Owner	Correct entry point is discoverable.
Service Discovery	Catalog item content and test categories guide correct selection.	Patient / Service Coordinator	Correct diagnostic service selected.
Test Configuration	Variables and dynamic behaviour create structured booking data.	Patient / Reception / IT Admin	Required data captured with minimal friction.
Submit & Validate	Validation checks patient data, slot availability and booking rules.	Patient / Platform Automation	Valid booking created without rework or conflict.
Approval & Governance	Flow Designer evaluates approval rules and records decisions.	Lab Manager / IT Admin	Decision recorded with audit evidence.
Test Fulfilment	Automation creates/assigns diagnostic tasks with patient context.	Lab / Diagnostic Team	Task completed with clear ownership.
Notifications & Status	Lifecycle notifications communicate appointment, sample and report changes.	Patient / Support	Status and notifications are timely.
Testing & Validation	Test evidence confirms booking, workflow, notifications, access and data accuracy.	Developer / Tester / Service Owner	Critical paths pass defined tests.
UAT & Acceptance	UAT feedback becomes controlled changes before production.	Stakeholder UAT Group	Acceptance criteria are satisfied.
Production Launch	Configuration is migrated and production behaviour is verified.	IT Admin / Release Owner	Production validation succeeds.
Monitor, Support & Improve	Operational data and feedback feed continuous improvement.	Support / Service Owner	Trend data drives improvement.

4. JOURNEY-TO-DESIGN BRIDGE

Customer experience	Simple discovery → guided booking → transparent approval → predictable test fulfilment → visible status → trusted report closure.
ServiceNow design	Service Catalog + Variables + Dynamic Behaviours + Validation + Flow Designer + Approvals + Tasks + Notifications + ACLs + Audit.
Delivery lifecycle	Design → Configure → Unit Test → End-to-End Test → UAT → Deploy → Monitor → Improve.
Evidence to capture	Booking data accuracy, slot validation, approval routing, task creation, notification delivery, report-status transitions, defect results, UAT evidence, production validation and operational metrics.

5. MAP OUTCOME

The Medical Diagnosis Center journey is designed as one continuous ServiceNow experience rather than a set of isolated configuration steps. The journey begins with the patient's need, moves through service discovery and booking, applies validation and governance, creates diagnostic fulfilment work, communicates status and reports, and ends with monitoring and continuous improvement.

This structure intentionally keeps each journey stage separate so that the document can be reviewed, edited and presented without overlapping text or compressed 11-column content.